



Providing Creative Solutions to Common Problems

HEALTHCARE & EMS RECEIVABLES · CAPABILITIES OVERVIEW

Debt Collection Agency for Businesses

Patient, payer & transport receivables recovery for EMS
and air-medical services

Prepared for **UT Health East Texas EMS & AIR 1** · July 2026

A recovery partner built for the East Texas EMS mission

To Nicole Willard and the UT Health East Texas EMS leadership team — you run East Texas' largest nonprofit EMS system: 65+ ambulances, four AIR 1 helicopters, and 80,000+ responses a year across Henderson, Cherokee, Smith and Wood counties. Every one of those responses generates a bill — and that is where the hardest part of the work begins.

The revenue rarely follows the response. EMS runs on some of the most difficult receivables in healthcare. A large share of patient transport accounts are effectively non-billable — wrong or stale contact information, no coverage on file, or a self-pay balance the patient cannot resolve on their own. Alongside them sit unresolved Medicare and commercial-payer claims that quietly age out of the recovery window.

Chasing those balances in-house pulls your billing staff off clean-claim work — and squarely into federal **consumer** collection law, because a patient balance is FDCPA- and Regulation F-governed debt, layered on top of your HIPAA obligations.

Our role is not to replace your billing office or your RCM vendor — it is to **extend** it. Through our contingency-only **Early-Out Program**, we work patient balances and open payer claims while they are still fresh and most recoverable, and carry the FDCPA, Reg F and HIPAA compliance load. You pay nothing unless we recover, and every contact is worked to protect the UT Health name in the communities you serve next call.

Two decades of recovery — with a spotless compliance record

A track record built on high-volume receivables across healthcare and commercial clients, worked under a compliance program designed for exactly the FDCPA, Reg F and HIPAA exposure EMS billing carries.

2004 Recovering receivables since	\$975M+ Receivables placed (all industries)	\$76M+ Recovered for clients	800K+ Accounts worked
20+ yrs Spotless FDCPA / HIPAA compliance record	15–60 days the early-out window — where recovery is highest	4.9★ Google client rating	\$0 upfront — pure contingency

HIPAA-compliant PHI handling

FDCPA / CFPB Reg F / FCRA compliant

Medicare & commercial-payer follow-up

Nationwide skip-tracing

Texas-based · Addison HQ

RECOGNIZED & FEATURED BY

[USA Today](#) [Financial Services Review — Top AR Management Service 2025](#)

[insideARM — 10× ROI](#) [CFO Tech Outlook — Top 10](#) [International Business Times](#)

ACCREDITED & RECOGNIZED



How we would support UT Health East Texas EMS

The questions a nonprofit EMS system should ask a recovery partner before placing a single account — answered directly.

WHAT YOU NEED	SOUTHWEST RECOVERY SERVICES
Fit for a large EMS & air-medical system	We work air and ground the same way: patient and responsible-party balances from every county you cover, plus open Medicare and commercial-payer claims — one partner, one process, one secure portal across your service area.
Is patient transport debt consumer debt?	Yes. When the balance sits with an individual patient or responsible party, it is FDCPA- and CFPB Reg F-governed debt: validation notices, documented dispute rights, contact-frequency limits and required disclosures on every account. Our agents are trained across that framework so your billing office does not carry the risk.
HIPAA & protected health information	Transport records are PHI. We apply the minimum-necessary standard, access only what the account requires, never discuss details with unauthorized parties, and exchange files through the secure portal — not email. Your HIPAA posture is preserved end to end.
Open Medicare & insurance claims	Unresolved payer claims are one of the largest sources of EMS write-offs. We identify open claims and pursue resolution with Medicare and commercial carriers as a distinct track, separate from patient contact — often the higher-yield lever.
Aged backlog (12–18 months)	Place it. We will give you an honest read on what is realistically recoverable, run skip-tracing on cold accounts, and work every file. You pay nothing on accounts that do not collect, so there is no cost to trying the backlog.
Fee structure & upfront cost	Pure contingency. No retainer, no setup fee, no monthly minimum. Our fee is a pre-agreed percentage of what we actually recover; if an account collects nothing, you owe nothing on it.
Timing & recovery	Recovery tracks with age. A patient account placed within 60–90 days of transport recovers meaningfully better than the same account at six months. We help you tighten the placement window on current accounts while we work the backlog.
Bad or missing contact information	The reason so many EMS accounts go non-billable. Nationwide skip-tracing to locate patients and responsible parties is standard on every placement, not an add-on.

WHAT YOU NEED	SOUTHWEST RECOVERY SERVICES
Reporting & account management	A dedicated account manager plus the secure client portal at swrsportal.com — real-time reporting, account status, secure PHI file exchange and online placement, 24/7, with role-based access built for a multi-facility health system. Detail below.
Protecting the UT Health name	Firm, effective, dignified contact — never the tactics that damage a community health system's reputation. The patient we speak with today is your neighbor, and your patient again tomorrow.

THE SWRS CLIENT
PORTAL

Full visibility on demand — built for a multi-facility health system

Every placement is managed through our secure client portal at swrsportal.com. Your billing office gets live, self-service access throughout the recovery process — backed by a dedicated account manager and role-based security suited to a HIPAA environment.

REAL-TIME ACTIVITY REPORTS

Run, filter by status and export account and recovery reports on demand — a live dashboard, not a monthly summary.

SECURE PHI FILE EXCHANGE

Send placements, statements and transport records safely inside the portal — never over unsecured email.

DIRECT-PAYMENT REPORTING

Log payments a patient makes to you directly, so balances stay accurate in real time and no one is over-contacted.

ACCOUNT STATUS & LOOKUP

Search any account for milestones, collector notes, payment history and clear status codes.

ONLINE PLACEMENT

Submit new accounts through structured forms, individually or in bulk, from any facility or billing desk.

ROLE-BASED ACCESS & MFA

Standard and manager roles with multi-factor security; managers move across accounts — built for a 10-hospital system.

► [Portal Video Tour](#)

Recover more, sooner — before accounts ever age

The single biggest lever on EMS receivables is timing. Our Early-Out Program works your balances in the early past-due window — roughly 15 to 60 days out — the highest-recovery stage there is: the balance is recent, the patient is still reachable, and a professional third-party notice often prompts payment before an account ever needs hard collections. It runs as a seamless extension of your billing office.



Validation & onboarding

Every account is run through address validation and NCOA, and screened for third-party eligibility and pending medical-assistance — so effort lands on good data from day one.



Notice series & professional contact

A customizable series of notices plus trained outbound calls over the early window — a neutral third-party voice while the balance is fresh, always within FDCPA / TCPA / Reg F where they apply.



Insurance & self-pay resolution

We surface additional insurance-billing opportunities, pursue verification, resubmission and appropriate appeals with Medicare and carriers, and set up self-pay arrangements patients can actually honor — all under HIPAA.



Reporting & hand-back

Weekly or monthly performance reporting through the secure portal. Anything unresolved in the early window is returned or escalated to full collections on your authorization — the full spectrum under one roof.

**WHY IT
WORKS**

Early placement is the difference between recovered and written off

Accounts get harder to collect in proportion to their age. By resolving balances while they are fresh, the Early-Out Program frees your billing staff for clean-claim work, accelerates cash flow, and — the real prize — sharply reduces the volume of EMS accounts that ever reach hard collections in the first place.

FOR YOUR TEAM

We carry the early follow-up and the FDCPA / Reg F / HIPAA compliance load, so your staff stays on billing, coding and patient service.

FOR YOUR CASH

Faster resolution, fewer write-offs, and a lower overall cost of recovery — because fewer accounts ever escalate.

SWRS Healthcare Case Studies

Actual placed-versus-recovered results from Southwest Recovery's healthcare portfolio, anonymized by sector. Recovery varies with account age, balance and payer mix — the results below span hospital, emergency-medicine, physician and post-acute clients.

COMMUNITY HOSPITAL SYSTEM

Multi-site acute care

\$1.46M recovered
of \$5.25M in placed patient
AR

28% recovered

SURGICAL HOSPITAL

Specialty facility

\$184K recovered
of ~\$446K placed

41% recovered

EMERGENCY-MEDICINE GROUP

Multi-site ER practice

\$269K recovered
of ~\$1.26M placed

21% recovered

REGIONAL HOSPITAL

Acute-care facility

\$250K recovered
of ~\$1.71M placed

15% recovered

PHYSICIAN GROUP

Multi-provider practice

\$143K recovered
of ~\$829K placed

17% recovered

SKILLED NURSING & CARE

Long-term care center

\$154K recovered
of ~\$877K placed

18% recovered

Anonymized by sector from Southwest Recovery's healthcare book; individual client names withheld. Results reflect full third-party placements and vary by account age, balance size, payer mix and contact-data quality. EMS self-pay recovers below facility averages — which is precisely why early placement and payer follow-up matter.

TEXAS · 2025–
2026

Why credit-report pressure no longer drives medical recovery

The rules changed. The CFPB moved in 2025 to strip most medical bills from consumer credit reports — an estimated **\$49 billion across roughly 15 million Americans** — a change since contested and narrowed in court. Ambulance and EMS balances are squarely medical debt, and were rarely reported to begin with. The practical effect: the credit report is no longer a meaningful lever on patient balances.

WHAT IT MEANS FOR RECOVERY

Recovery now depends on reaching the right party early, resolving insurance and eligibility, and arranging payment a patient can honor — not the threat of a credit hit. That is exactly what our Early-Out program is built to do.

THE TEXAS CLOCK

Texas sets a **four-year statute of limitations** on this debt, and it cannot be revived by a payment. The window to recover is finite — another reason to place accounts while they are fresh rather than let them age.

Because credit reporting is voluntary and now largely off the table for medical debt, the levers that still work — early contact, skip-tracing, and payer follow-up — are the ones we lead with.

\$49_B

medical debt slated to leave U.S. credit reports (CFPB, 2025)

15_M

Americans affected by the change

4 yrs

Texas statute of limitations on this debt

<\$500

typical reported medical-debt balance

Sources: CFPB medical-debt reporting rule (2025) and Texas debt-collection statute of limitations. Figures are context for recovery strategy, not legal advice.

The economics — recover more at no fixed cost

Whether to expand an internal billing desk or place with a specialist comes down to unit cost and reach. A contingency partner working accounts early changes the math — without your system adding a single billing seat.

Right-party contact on aged accounts

Internal

18–25%

SW

Recovery

35–48%

Recovery worked on two fronts

Patient + payer

Self-pay balances *and* open Medicare / insurance claims — internal desks rarely chase the claim side, where much of the money sits.

Your fixed cost to run it

\$0 fixed

Pure contingency — a share of what we actually recover, never a salary line that runs whether accounts collect or not.

IN-HOUSE BILLING DESK

Fixed cost

Salary, benefits, training and compliance risk — paid whether or not the account collects.

SOUTHWEST RECOVERY

\$0 unless we collect

Contingency only, on dollars actually recovered — and the FDCPA / Reg F / HIPAA risk moves to us.

Placed early through the Early-Out Program, more of every dollar comes back — and fewer accounts ever reach hard collections.

We do not promise a fixed recovery percentage, and we are wary of any agency that does. Early placement consistently outperforms late placement; we will give you an honest read on a representative sample of your accounts before any are worked.

We speak your billing

High-volume transport billing across a wide service area creates a specific receivables profile. Our recovery playbook is built for exactly that.



Non-billable & cold-contact accounts

The 40%+ of EMS accounts that go non-billable are largely a contact-data problem. Nationwide skip-tracing to locate patients and responsible parties is standard on every placement.



Open Medicare & payer claims

A major, often-overlooked source of EMS write-offs. We pursue unresolved claims with carriers as a dedicated track, separate from patient contact.



Aged self-pay & workable plans

Older balances resolve when a professional agency reaches the patient with a clear option they can actually honor. We work every file and tell you upfront what is realistic.



Air + ground, one portal

AIR 1 and 65+ ambulances across four counties, managed as one program — role-based portal access and per-facility reporting for a multi-hospital system.

CLEAR LANES**Where we work — and where your team and counsel stay**

We are built to slot in alongside your billing office, your RCM processes and your counsel — not to step on any of them. The lanes stay clean.

WHAT WE DO

Work the aged patient balances and open payer claims you place, under the SWRS FDCPA / Reg F / HIPAA compliance umbrella — skip-tracing, documented contact, carrier follow-up and payment arrangements — all on contingency, at no cost unless we recover.

WHERE YOUR TEAM & COUNSEL STAY

Front-end billing, coding and clean-claim submission remain with your office and RCM vendor. Anything that calls for litigation or a legal remedy stays with your counsel; we hand off an organized, fully documented file and keep working recovery alongside them so nothing stalls.

We track the evolving federal and Texas rules on ambulance balance billing and medical-debt credit reporting, so your recovery program stays compliant as those rules change.

Regardless of the means of recovery, our focus is always on preserving goodwill and representing the client professionally.

— Steven Dietz, Founder & Chief Executive Officer, Southwest Recovery Services

A community EMS system does not get to treat its patients as strangers. The person on the other end of a transport bill is a neighbor in Tyler or Athens or Rusk — and very likely a future patient of UT Health. Tone matters as much as outcome. We apply firm, effective, dignified pressure without the tactics that burn a healthcare relationship or read as a generic collections call — recovery that never reflects poorly on the UT Health name.

Take it with you

Download this overview and the supporting compliance white papers to share with your billing, finance and compliance teams.



This capabilities overview

PDF · prepared for UT Health
East Texas EMS

[Download PDF ↓](#)



Medical Debt Collection Compliance

White paper · PDF

[Download PDF ↓](#)



Why Early Placement Recovers More

White paper · EMS & healthcare

[Download PDF ↓](#)

Let's find what's recoverable.

A 30-minute discovery call to walk your two tracks — aged patient balances and open payer claims — and set an honest expected recovery range against a representative sample of your accounts.

[Book a 30-minute discovery call →](#)

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Private & confidential — prepared for UT Health East Texas EMS & AIR 1. FDCPA / CFPB / FCRA compliant · HIPAA-aware PHI handling · ACA International member.
SWRS does not collect consumer debt in CA, OR or WA.